Obstetric Ultrasound / Morphology

Date: 01 Jan 2026

Patient	FICTIONAL PATIENT	**Identifier**	DEMO-001

Examination context

Indication and examination type: Routine mid-trimester anatomy assessment (fictional).

Technique and image quality: Transabdominal examination; some views limited by fetal position.

Established gestational age, EDD and dating basis: 20 weeks 2 days; EDD 19 May 2026, based on an earlier dating scan (fictional).

Number of fetuses: 1

Chorionicity / amnionicity if multiple: Not applicable: singleton.

Fetal identifier and presentation: Fetus A; longitudinal lie.

Biometry

Biparietal diameter (BPD): 47 mm

Head circumference (HC): 175 mm

Abdominal circumference (AC): 150 mm

Femur length (FL): 32 mm

Estimated fetal weight: 330 g

Formula, chart, centile and comparison: Demonstration measurements only; centile not assigned. Record the validated formula/chart used.

Fetal heart rate: 148 beats/min

Anatomy assessed

Cranium, brain and face: Cranium and standard brain views visualised; profile view incomplete.

Spine: Visualised in the obtained planes.

Thorax and cardiac views: Four-chamber view obtained; outflow views incomplete.

Abdominal wall, stomach and cord insertion: Visualised in the obtained views.

Kidneys and urinary bladder: Both kidneys and bladder visualised.

Limbs: Four limbs visualised; detailed assessment documented in image record.

Placenta and maternal observations

Placental location and relationship to cervix: Anterior placenta; relationship to internal os documented on images.

Amniotic fluid assessment and method: Deepest vertical pocket 4.2 cm.

Cervix: assessment method and findings: Formal transvaginal cervical length not performed.

Maternal uterus / adnexa and other findings: No additional finding documented in this example.

Conclusion

Incomplete views and limitations: Profile and cardiac outflow views not adequately assessed.

Impression: Single live fetus; anatomy assessment incomplete as above. Measurements alone do not exclude abnormalities.

Follow-up and communication: Arrange completion of unassessed views according to the responsible service protocol.

Clinician: DEMONSTRATION ONLY - NOT SIGNED **Signature:** _____